



Occupational Medicine & Insurance

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- What does OH do?
- OH view on managing people back into the workplace
- What interventions work and why? (Thinking about CBT, physio etc).
- Psychological overlay
- What are the challenges that OH faces from the patients and employers point of view.
- How can the insurance industry work with OH?



- A branch of clinical medicine
- Concerned with keeping people well at work – both physically & psychologically
- Workplace safety and the prevention of occupational injuries and disease
- Assessing fitness for work
- Helping to rehabilitate those who have suffered injury or sickness back into work
- Management of sickness absence



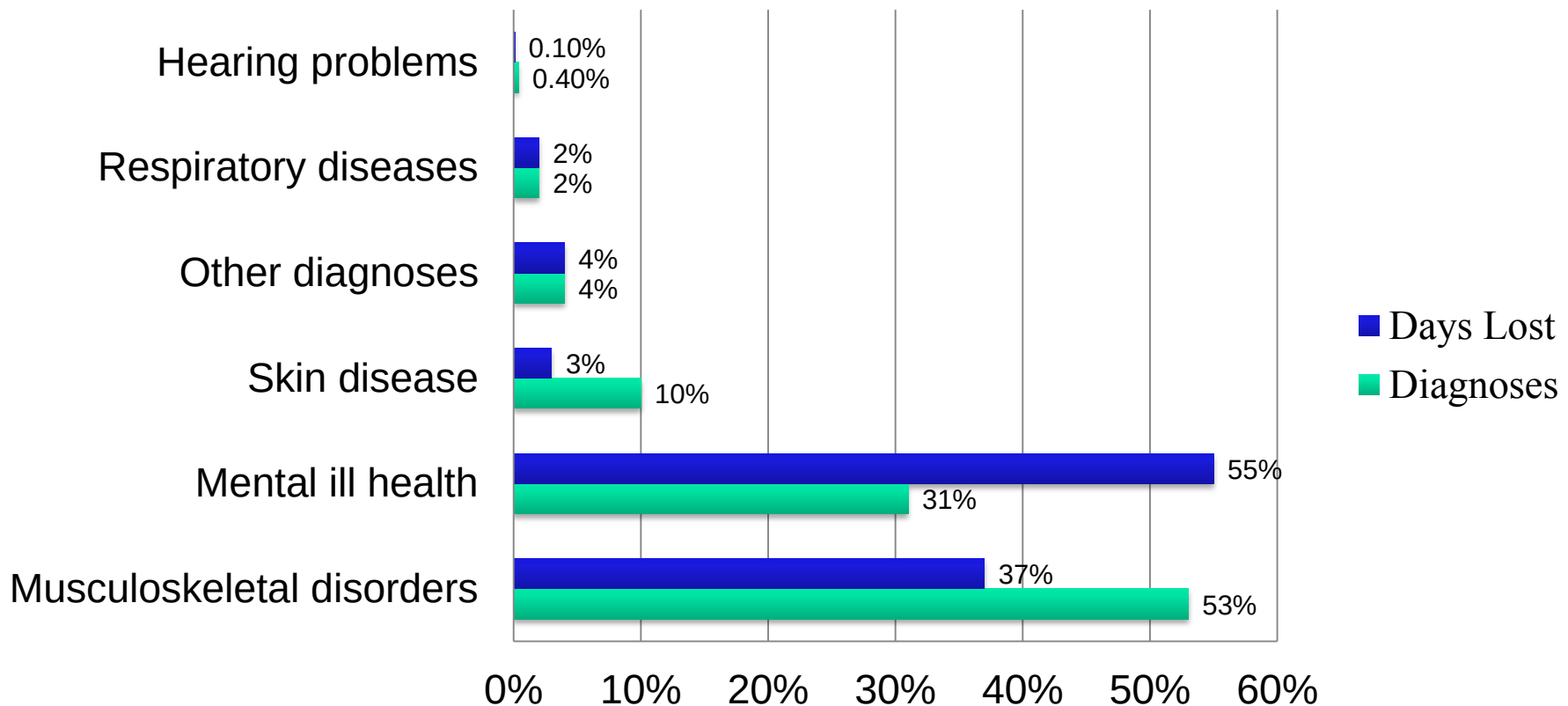
1.3m people in work are estimated to have suffered from work-related ill health in GB 2009/2010

- 572,000 musculoskeletal disorders
- 435,000 stress, depression, anxiety
- 38,000 respiratory

<http://www.hse.gov.uk/statistics/overall/hssh0910.pdf>

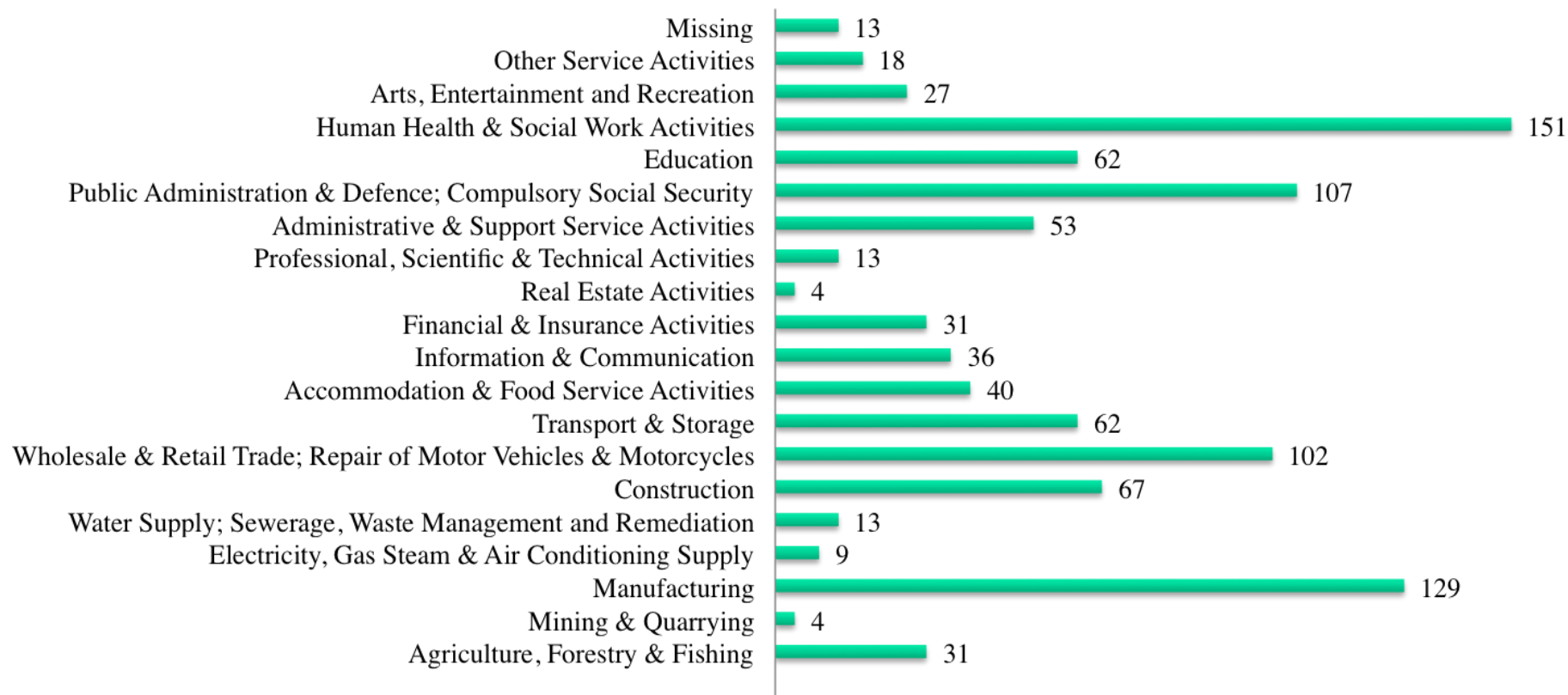


Proportion of days lost by diagnosis as reported by GP's for 2007-9





THOR: Total estimated cases of occupational diseases 2012 (weighted)*



Getting back to work



- Work is generally good for you
- Short absences shouldn't require prolonged rehabilitation
- Fitness for work is not an absolute and non-medical factors are often more important than medical ones in preventing return to work.
- Return to work should not be delayed until recovery is complete



- Beliefs
 - Work made me ill
 - If I go back I'll get worse
- Upbringing/personality
 - Overprotective parents
 - Avoidant personality
- Psychological
 - Depression
 - Anxiety
 - Catastrophisation
- Social
 - What colleagues, friends & family say
 - Attitude of employers
- Delay in diagnosis/treatment
- Harmful medical advice



- Timely ones
 - Not too early or late
- Appropriate ones
 - Don't over-medicalise (CBT vs counseling)
- Work focus rather than just symptom relief
- Early physiotherapy
- CBT, but be selective
- Graded return to work and work visits while off sick



- Co-morbidity is possibly a better term
- Almost universal to some degree
- Fear avoidance
- Catastrophisation
- Depression is common and under-diagnosed after life-changing illnesses (CVA, cancer, coronary thrombosis)
- Inactivity and being off sick is depressing



- Consent to share information
- Case conferences (depending in size of employer, regular or ad hoc)
- OH can recommend interventions (CBT, physio)
- A good OH report should not usually say fit/unfit
- OH report should distinguish between what is known and what is told (objective/subjective)
- OH can identify obstacles (medical and other) to return to work and ways to remove them
- A sensible OP will not comment on surveillance other than to suggest that the insurer weigh up the merits of all evidence in their possession.



- Much smaller industrial base in UK
 - Coalmining
 - Steel
 - Shipbuilding
- Maintaining relevance in changing environment
- Moving away from risk-based assessments (Stress)
- Proving value
- Complaints culture